

**THE PLAN**

Where you stand

An honest look at what exists today, what is missing, and what to fix first.

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

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VERSION

1.0 · Confidential draft

An honest look at what exists today, what is missing, and what to fix first.

In one minute

- Everything that exists today is an idea, a name, a logo, one prototype and one conversation that went nowhere. That is the true starting point.
- The business scores **11 out of 100**. Under 20 means a company that has not started trading.
- Ten problems are listed below. The first four block everything else and take about four weeks to clear.
- Ten opportunities are listed too. The best one is not the product. It is selling your own advice while the product is built.
- Nothing here is a criticism of the idea. The idea is sound. The company around it does not exist yet.

What to do

Action	Who	By when
Agree in writing who owns MaternalLink	You and David Agunede	Day 3
Register the company after checking the name	You	Day 10
Withdraw the 2025 AI deck and lock the prototype	You	Day 3
Book a code and security check of the prototype	A developer you pay	Day 14
Re-score this document	You	14 October

What was actually reviewed

What	Where it was	What it is	Verdict
The code repository	GitHub	One file with one line of text	Empty
Your written brief	This project	A clear, complete description of the company you want to build	The strongest thing you had
"Black Maternal health" deck, October 2025	Google Drive	13 slides proposing AI that predicts pre-eclampsia, sepsis, bleeding and blood clots	An idea only. No data, no model, no clinician, no permission to use patient data
Ekiti State proposal, March 2026	Email	Sent by David Agunede as "Founder and CEO, Maternal Link". £4.5m for 15,000 women, rising to £36m	A proposal with no reply. The price is far above anything comparable
The prototype dashboard	maternal-health-psi.vercel.app	A working website with patients, alerts and a risk engine that suggests clinical actions	Real, but nobody has checked who owns the code or whether it is secure
Kemek Enterprise investor files	Google Drive	Your separate property business, with its own 128-name investor list	Not Vytalix. It shows you can run a disciplined process. It also splits your time
Public record for "Vytalix"	Web search	No company found. Similar names exist in the UK and the US	The name is at risk until checked properly

Nothing else was found. No business plan, no financial model, no website, no brand, no contracts, no customers, no revenue, no grants, no partners, no clinicians, no policies, no insurance.

The score, area by area

Scoring: 0 means nothing exists. 3 means an idea or a draft. 5 means a usable draft that nobody has tested. 7 means it works and there is proof. 10 means best in class.

Area	Score	Why
Company and governance	0	No registered company, no board, no shareholder agreement
Name and brand	1	A logo exists. The name has not been checked and clashes with others
Business model and prices	1	One price existed and it was wrong
Advisory and consulting	1	An idea. No offers, no collateral, no clients. The fastest money in the company
MaternaLink	2	A prototype and a concept. No clinician, no evidence, no plan for regulation
Clinical safety	0	No safety officer, no hazard log, no statement of what the product is for
Data protection and regulation	0	Not registered with the ICO. No data protection assessment. No thinking about medical device rules
Who owns the intellectual property	0	Nothing written down with David Agunede or with whoever wrote the code
Technology	1	One prototype of unknown quality. No domain, no email, no cloud account
E-learning	0	Nothing exists
Proof that customers want it	1	One meeting in Nigeria. No UK conversations at all
Knowing the competition	1	The Ekiti proposal did not mention that the state already has a funded programme
Partners	1	One ministry meeting. No agreements
Sales system	0	No contact list, no scripts, no proposals
Marketing	0	No website, no company page, nothing published
Money and accounts	0	No model, no budget, no bank account, no accountant
Ready for investors	1	You have shown you can run a list. Vytalix has no deck, no data room, no tax clearance
Team	1	One founder and one informal collaborator with unclear roles
Routines and reporting	0	None
Credibility: advisers, evidence, press	0	None
Total	11 / 100	Pre-formation

What you have, what you are building, what comes next

Say it in this order to investors. Never inflate the first column.

What you have

- A full company plan
- A prototype (needs an audit)
- A past government conversation
- A logo and two websites

Building now

- The company itself
- Ownership agreement
- First advisory clients
- A hospital trial

Next

- First paying customer
- A signed pilot
- Advisers on board
- Pre-seed round

Say it in this order to anyone who asks. Never inflate the first column.

What is strong and what is weak

Strong. A precise founder brief. A genuinely important problem with real political attention behind it in the UK. A live conversation in Nigeria and a working prototype, which most founders at this stage do not have. Proof that you can run an investor list properly.

Weak. No company, no ownership, no brand protection. Medical claims with nothing behind them. A price built on nothing. One founder doing everything, across two businesses. No web presence at all.

The chances. Selling your own advice pays within weeks. A re-scoped MaternaLink can be trialled without becoming a medical device. Grants suit this story. Women's health and impact investors are actively looking for something like it. Partnering with the Nigerian incumbent turns a threat into a route in.

The threats. A name clash with two US health-tech companies. Regulatory trouble if any prediction feature ships. A falling-out over who owns MaternaLink. Slow, political government sales in Nigeria. An investor finding all of this in ten minutes.

The ten problems, worst first

#	Problem	Why it matters	What fixes it	By
1	Nobody owns MaternaLink on paper	Blocks funding, contracts and trials. Risks a dispute	Meet this week, agree heads of terms, then a solicitor drafts a deed	Day 7
2	No company	Cannot contract, bank, apply for grants or take investment	Check the name, then register	Day 10
3	Medical claims with no evidence	Regulatory and reputational risk. Fails any investor check	Withdraw the deck. Re-scope the product. Rewrite what it does	Day 3
4	The name may not be yours	Two US health-tech companies use near-identical names	Trademark searches, then file	Day 20
5	No money coming in	Your time is the only funding	Launch three advisory offers, hold 40 conversations	Day 45
6	The £300 per woman price	Anyone who knows the market will see it is wrong	Replace with the new price book	Day 21

#	Problem	Why it matters	What fixes it	By
7	No clinician anywhere near this	You cannot build maternity software without midwives and doctors	Recruit four advisers	Day 60
8	The prototype is unchecked	May contain insecure code, other people's work, or real patient data	Pay for a code and security audit	Day 14
9	Nobody in the UK has been asked	Your home market is untested	15 interviews with midwives and maternity leads	Day 60
10	Two businesses, one of you	Everything waits for you	Write down your time split. Get help with admin	Day 14

The ten opportunities, best first

#	Opportunity	Why it is good	First step
1	Sell your own advice	Cash in weeks, references, credibility	Publish three offers, start 40 conversations
2	MaternaLink as a care-coordination service	The product story, without the medical device problem	Write what it is for, get one clinician to agree
3	Work with the Nigerian incumbent, not against	Keeps the government relationship alive	Reply to Ekiti offering to complement, not replace
4	Grants	£50,000 to £500,000 without giving away shares	Check the Innovate UK and SBRI Healthcare calendars
5	Women's health and impact investors	Better terms and aligned expectations	Work the A-list after the blockers are cleared
6	Publish one research report	Authority, press, inbound interest	Outline it in week five
7	Training for midwives and health workers	Repeat income, low regulatory risk	Two pilot modules with an NGO
8	Consulting for companies entering the NHS or Africa	High margin, repeatable	Package two fixed-price sprints
9	A university research partnership	Turns the 2025 AI idea into fundable research	Approach a maternal health research group
10	Telecoms and insurers in Nigeria	Reach without government budgets	Keep on the partner list for year two

What creates value, and what creates risk

Brings money in now	Builds credibility	Makes investors comfortable	Creates real risk
Advisory days, consulting projects, grant-funded work	Advisers, a published report, a real trial, a professional website	A registered company, clean ownership, sensible prices, a named pipeline	Prediction claims, health data without safeguards, using the prototype with real patients, unclear purpose

Words explained

Word	What it means
Heads of terms	A short written summary of what two people have agreed, before the lawyers write the full contract
ICO	The Information Commissioner's Office, the UK regulator you must register with to handle personal data
Data protection impact assessment	A written check of what could go wrong with people's data, required before you handle health information
Hazard log	A list a health software team keeps of everything that could harm a patient, and what stops it
Medical device	Anything, including software, whose purpose is to diagnose, predict or treat. Tightly regulated